

HOSPITAL DISCHARGE SUMMARY

PATIENT INFORMATION

Name: Patient 44

Age: 81 years old | Sex: Female

MRN: 000-TEST-44

Admission Date: [Admission Date] | Discharge Date: [Discharge Date]

Attending Physician: Dr. [Attending Name]

PRIMARY DIAGNOSIS

Acute appendicitis with perforation s/p laparoscopic appendectomy with intraoperative conversion to open procedure

SECONDARY DIAGNOSES

- Vascular dementia (moderate)
- Osteoporosis with history of L2 vertebral compression fracture
- Hypertension
- Type 2 Diabetes Mellitus
- Hyperlipidemia

HOSPITAL COURSE

81-year-old female admitted via ED with acute abdomen and imaging studies confirming perforated appendicitis. Initial laparoscopic approach was attempted with three 10-12mm trocars placed at umbilicus, suprapubic, and left lower quadrant. Intraoperative findings revealed extensive peritoneal contamination and dense adhesions; procedure converted to open midline laparotomy for adequate visualization and specimen removal. Appendectomy completed without complications. Peritoneal lavage performed with 3L NS. Single running closure of fascia with absorbable suture; skin closed with staples. EBL 150mL. Patient tolerated procedure well, extubated without difficulty.

Post-op course was uncomplicated. Nasogastric tube removed POD #2. Patient transitioned from NPO to clear liquids on POD #2, advanced to regular diet as tolerated on POD #3. Pain

controlled with opioid analgesics and acetaminophen. Foley catheter discontinued POD #2 with appropriate urinary function. No evidence of SSI or other post-operative complications. Ambulation progressed per PT; patient discharged to home with family support given dementia diagnosis.

DISCHARGE MEDICATIONS

| Medication | Dose | Route | Frequency | Duration |

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| Acetaminophen | 650 mg | PO | q.6h PRN pain/fever | 2 weeks |

| Hydrocodone/APAP | 5/325 mg | PO | q.4-6h PRN moderate-severe pain | 1 week then d/c |

| Metformin | 500 mg | PO | BID | Continue ongoing |

| Lisinopril | 10 mg | PO | q.d. | Continue ongoing |

| Atorvastatin | 40 mg | PO | q.h.s. | Continue ongoing |

| Vitamin D3 + Calcium | 1200 mg Ca/800 IU D | PO | BID | Continue indefinitely |

WOUND CARE & RESTRICTIONS

Three small laparoscopic trocar sites (umbilicus, suprapubic, LLQ) require daily cleaning with soap and water; keep dry. Midline incision (approximately 5-6cm) may have staples or skin closure strips; do NOT submerge in water until cleared by surgeon. Apply dry sterile dressing if drainage present. Report any signs of infection (increasing erythema, purulent drainage, fever >101°F).

NO LIFTING > 5 lbs for 6 weeks. No strenuous activity, heavy housework, or driving while on narcotic medications. Gradual return to normal ADLs as tolerated. Osteoporosis precautions: avoid bending at waist, use proper body mechanics.

DIET

Clear liquids × 24-48 hours post-discharge if tolerated. Advance to regular diet; small frequent meals preferred. Avoid high-fat foods initially. Resume diabetic diet per home glucose monitoring.

FOLLOW-UP APPOINTMENTS

1. Surgery clinic — in 2 weeks for incision check & staple removal